

Department of Nephrology
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 40.4 Yr/M
Admission No.: anonymous	Department Unit: Nephrology (Nephro Unit 1)	Consultant/Faculty: anonymous
Ward: 3 C 1 Nephrology ward	Room/Bed: 3 C 1 Nephrology/ 3C1 NP 9	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 31-Mar-2023 13:27	D.O.D. : 04-Apr-2023	D.O. Print Report: 08-Feb-2026 15:15

FINAL DIAGNOSIS:

CKD-5D(CGN)

HTN

ANAEMIA

CHIEF COMPLAINTS:

VOMITING,DECREASED APPETITE.

HISTORY OF PRESENT ILLNESS: VOMITING X 7 DAYS,DECREASED APPETITE.

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
31-Mar-2023 18:10	ACTIVATED PARTIAL THROMBOPLASTIN TIME (APTT)	APTT HEADER	23.0	23.2 - 32.4 sec
		APTT NOTE	Refer Reports*	-
		clinical details	--	-
	Calcium	Calcium	8.9	08.7 - 10.4 mg/dL
	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.02	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.60	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	2.46	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	0.25	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	6.77	2 - 7 x10 ³ /μL
		Basophil	0.2	00 - 01 %
		Eosinophils	5.9	01 - 06 %
		HCT	24.3	40 - 50 %
		Hemoglobin	7.5	13 - 17 g/dl
		Lymphocytes	24.4	20 - 40 %
		MCH	24.0	27 - 32 pg

	MCHC	30.9	31.5 - 34.5 g/dL
	MCV	77.9	83 - 101 fL
	Monocytes	2.5	2 - 10 %
	Neutrophils	67.0	40 - 80 %
	Platelet Count	225	150 - 410 x10 ³ /μL
	RBC	3.12	4.5 - 5.5 x10 ⁶ /μL
	Red Cell Distribution Width (RDW CV)	14.3	11.6 - 14.0 %
	Total Leukocyte Count(TLC)	10.10	4 - 10 x10 ³ /μL
HBsAg (ELISA)	Interpretation	Non Reactive	-
	Result	Refer Reports*	-
HCV Antibody (ELISA)	Interpretation	Non Reactive	-
	Result	Refer Reports*	-
IONISED CALCIUM	Ionized Calcium	1.22	1.12 - 1.32 mmol/L
Liver Fuction Test (Lft)	Albumin	3.1	4.0 - 5.0 gm/dL
	ALP	59	50 - 116 U/L
	ALT	32	7 - 41 U/L
	AST	17	12 - 38 U/L
	Direct Bilirubin	0.08	0.1 - 0.4 mg/dl
	globulin	2.8	2.0 - 3.5 gm/dL
	Indirect Bilirubin	0.37	0.20 - 0.90 mg/dl
	Total Bilirubin	0.45	0.3 - 1.3 mg/dl
	Total Protein	5.9	6.7 - 8.6 gm/dL
PHOSPHOROUS	Phosphorous	>9	2.8 - 4.7 mg/dL
PROTHROMBIN TIME (PT INR)	clinical details	--	-
	International Normalised Ratio (INR)	1.1	-
	PT INR Header	10.8	09.6 - 12.2 sec
	PT INR Note	Refer Reports*	-
Rft	Chloride	101	102 - 109 mmol/L
	Creatinine	14.54	0.9 - 1.3 mg/dl
	Potassium	5.02	3.5 - 5 mmol/L
	Sodium	136	135 - 145 mmol/L
	Urea	211	15.0 - 39.0 mg/dl
URINE ROUTINE AND MICROSCOPIC EXAMINATION (R/M)	Appearance	CLEAR	-
	Bilirubin	NOT DETECTED	-
	Casts	NOT DETECTED	-
	Colour	PALE YELLOW	-



Comments	Refer Reports*	
Crystals	NOT DETECTED	-
Epithelial Cells	NOT DETECTED	0 - 5 / hpf
Glucose	NOT DETECTED	-
Ketones	NOT DETECTED	-
Nitrite	NEGATIVE	-
Others	NOT DETECTED	-
Protein	NOT DETECTED	-
Pus cells	OCCASIONAL	-
RBC	NOT DETECTED	-
Reaction pH	6.0	5 - 7 .
Specific Gravity	1.020	1.003 - 1.035 .
Urobilinogen	NORMAL	-

***Refer Lab Reports for details.**

HOSPITAL COURSE: PT P/W UREMIA AND VOLUME OVERLOAD. INITIALLY DIALYSED VIA RT FEMORAL UTCC. PATIENT WAS PLANNED FOR PERM CATH INSERTION FOR CONTINUATION OF MHD. PERM CATH(TCC) INSERTION WAS DONE UNDER ALL ASEPTIC PRECAUTIONS AND WITH PROPER CONSENT OF PATIENT. POST TCC INSERTION CXR SHOWED PROPER PLACEMENT OF TCC. HEMODIALYSIS WAS INITIATED VIA TCC (RIGHT IJV) AND ONE SESSION OF HD WAS COMPLETED SUCCESSFULLY. PATIENT WAS COUNSELLED FOR AV FISTULA SURGERY. PRESENTLY, PATIENT IS STABLE AND IS BEING DISCHARGED ON MHD.

CONDITION AT DISCHARGE: STABLE

ADVICE ON DISCHARGE:

T.DYTOR 40 PO BD
T.PANTOP 40 PO OD BBF
T.IFA PO OD
INJ EPO 10 KIU S/C ,ONCE WEEKLY.NEXT DOSE ON 7/4/23
T.NICARDIA R 20 PO TDS
T.PRAZOCIN 5 PO BD
T.ARKAMIN 0.1 PO TDS
T.SHELCAL 500 PO OD
T.SEVELAMER 400 PO TDS

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 17-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : FOLLOW UP NEPHROLOGY OPD WITH FRESH REPORTS CBC,LFT,RFT.

Follow Up in Urology/CTVS/CENTRE OF CHOICE OPD for AV fistula surgery.

FOLLOW UP IN CFM OPD FOR VACCINATION OF HEPB,INFLUENZA,PCV.IF NOT ALREADY DONE

HD 2 TIMES PER WEEK AT CENTRE OF CHOICE

Summary Prepared By

anonymous

Summary Approved By

anonymous